



Bowel: Ischemia, Malabsorption & Polyps

Pathology GIT Block · MEDucation by Mattin Majid · Practice MCQs

HOW TO USE THIS SET

20 questions, mixed difficulty, 10 case-based. Attempt every question before checking the answer key on the last page.

● 4 Easy

● 10 Medium

● 6 Hard

1.**EASY**

Ischemic bowel disease is mainly due to impairment of:

- A.** Arterial supply
- B.** Venous drainage
- C.** Innervation
- D.** Lymphatic drainage
- E.** Bile flow

2.**EASY**

The treatment of celiac disease is:

- A.** Vitamin K alone
- B.** Iron infusion
- C.** A gluten-free diet
- D.** Colectomy
- E.** Lactose restriction

3.**EASY**

Intestinal polyps are most common in the:

- A.** Colon
- B.** Duodenum
- C.** Esophagus
- D.** Jejunum
- E.** Ileum

4.**EASY**

Bleeding and purpura in malabsorption reflect deficiency of:

- A.** Albumin
- B.** Vitamin K
- C.** Folate
- D.** Vitamin B12
- E.** Magnesium

5.**MEDIUM****CASE**

A hypovolemic patient has abdominal pain and bloody diarrhea after a hemorrhage.

Which bowel regions are most liable to ischemia in this setting?

- A.** Splenic flexure and sigmoid colon
- B.** Cecum and appendix
- C.** Rectum and anal canal
- D.** Duodenum and jejunum
- E.** Hepatic flexure and transverse colon

6.

MEDIUM

Ischemic injury confined to the mucosa and submucosa is called:

- A. Watershed infarction
- B. Serosal necrosis
- C. Mural infarction
- D. Transmural infarction
- E. Mucosal infarction

7.

MEDIUM

CASE

A 30-year-old has diarrhea, flatulence, weight loss and fatigue. The pathologist requests a small bowel biopsy.

Where is the diagnostic biopsy usually taken from?

- A. Sigmoid colon
- B. Gastric antrum
- C. Second portion of the duodenum or proximal jejunum
- D. Terminal ileum
- E. Rectum

8.

MEDIUM

CASE

A duodenal biopsy shows villous atrophy.

Which two other histological features complete the celiac triad?

- A. Granulomas and crypt abscesses
- B. Increased goblet cells and crypt atrophy
- C. Neutrophilic cryptitis and pseudopolyps
- D. Increased intraepithelial CD8+ lymphocytes and crypt hyperplasia
- E. PAS-positive macrophages and dysplasia

9.

MEDIUM

Which HLA types are associated with celiac disease?

- A. HLA-DR3 only
- B. HLA-B51
- C. HLA-DQ2 or HLA-DQ8
- D. HLA-B27
- E. HLA-A2

10.

MEDIUM

CASE

A patient with steatorrhea and weight loss develops tetany and osteopenia.

Which deficiencies explain these findings?

- A. Vitamin B12
- B. Vitamin K
- C. Iron and folate
- D. Calcium, magnesium and vitamin D
- E. Albumin

11.

MEDIUM

CASE

A toddler has failure to thrive and diarrhea. Serology is planned.

Which antibodies are diagnostically useful in the suspected disease?

- A. Rheumatoid factor
- B. Anti-tTG, anti-deamidated gliadin and anti-endomysial antibodies
- C. Anti-neutrophil cytoplasmic antibodies
- D. Anti-mitochondrial antibodies
- E. Anti-nuclear antibodies only

12.

MEDIUM

Which is the triad of solitary rectal ulcer syndrome?

- A. Polyps, pigmentation, family history
- B. Diarrhea, dermatitis, dementia
- C. Pain, fever, leukocytosis
- D. Steatorrhea, weight loss, anemia
- E. Rectal bleeding, mucus discharge, inflammatory lesion of the anterior rectal wall

13.

MEDIUM

Which is the commonest type of hamartomatous polyp?

- A. Juvenile polyp
- B. Peutz-Jeghers polyp
- C. Hyperplastic polyp
- D. Inflammatory polyp
- E. Tubular adenoma

14.

MEDIUM

CASE

A 65-year-old has a colonic polyp whose biopsy shows serrated, sawtooth glands limited to the upper half of the crypts, with no conventional dysplasia.

Which statement about this polyp is correct?

- A. It is a villous adenoma with high invasive potential
- B. It is hyperplastic; its risk of progression to malignancy is questionable
- C. It is a juvenile polyp
- D. It is defined by epithelial dysplasia
- E. It is a Peutz-Jeghers polyp

15.

HARD

CASE

A young patient has dark mucocutaneous pigmentation around the lips and multiple small-bowel polyps that show an arborizing (tree-like) network of smooth muscle and glands.

Which statement is correct?

- A. It is caused by impaired anorectal relaxation
- B. The polyps are typically confined to the rectum
- C. The polyps are adenomas defined by dysplasia
- D. It is autosomal recessive with no cancer risk
- E. It is autosomal dominant with increased risk of colon, pancreas, breast and other malignancies

16.

HARD

Which is NOT listed as an association of celiac disease?

- A. Ulcerative colitis
- B. Dermatitis herpetiformis
- C. Small intestinal adenocarcinoma
- D. Splenic atrophy
- E. Enteropathy-associated T-cell lymphoma

17.

HARD

Which malabsorptive disease shows a defect of lymphatic transport only?

- A. Abetalipoproteinemia
- B. Chronic pancreatitis
- C. Celiac disease
- D. Whipple disease
- E. Cystic fibrosis

18.

HARD CASE

A patient has malabsorption caused solely by a defect of transepithelial transport.

Which disease fits this pattern on the slide grid?

- A. Whipple disease
- B. Abetalipoproteinemia
- C. Celiac disease
- D. Chronic pancreatitis
- E. Cystic fibrosis

19.

HARD CASE

A 52-year-old at screening colonoscopy has a 1.2 cm pedunculated polyp. Histology shows epithelial dysplasia.

Which statement is correct?

- A. It is a hamartoma
- B. It is a juvenile polyp
- C. It is a hyperplastic polyp with no malignant potential
- D. All such lesions progress to carcinoma
- E. It is a benign adenoma; adenomas give rise to most colorectal adenocarcinomas, but not all progress

20.

HARD CASE

A large sessile colonic polyp is covered by slender villi.

Which adenoma type is this, and what is its distinguishing risk?

- A. Tubular; small and pedunculated with no cancer risk
- B. Tubular; more frequently contains invasive malignancy
- C. Tubulovillous; always benign
- D. Villous; more frequently contains foci of invasive malignancy
- E. Hyperplastic; sawtooth crypts

ANSWER KEY & RATIONALE

Don't peek until you've committed to an answer on every question.

Q	Ans	Why
1	A	Mainly impaired arterial supply; impaired venous drainage is uncommon.
2	C	Treatment of celiac disease is a gluten-free diet.
3	A	Intestinal polyps are most common in the colon.
4	B	Bleeding/purpura = vitamin K. Anemia = iron, folate, B6, B12; osteopenia/tetany = Ca, Mg, vitamin D; edema = albumin.
5	A	Non-obstructive hypoperfusion (e.g. hypovolemic shock) hits the watershed zones: splenic flexure and sigmoid colon.
6	C	Mucosal (a); mural = mucosa and submucosa (b); transmural = all three layers (c).
7	C	Biopsies from the second portion of the duodenum or proximal jejunum are generally diagnostic in celiac disease.
8	D	Celiac histology: increased intraepithelial CD8+ T lymphocytes, crypt hyperplasia, villous atrophy.
9	C	Gliadin, deamidated by tTG, interacts with HLA-DQ2 or HLA-DQ8 on antigen-presenting cells.
10	D	Osteopenia and tetany reflect deficiency of calcium, magnesium and vitamin D.
11	B	In celiac disease the B cell response produces anti-tissue transglutaminase, anti-deamidated gliadin and anti-endomysial antibodies.
12	E	The triad is rectal bleeding, mucus discharge and an inflammatory lesion of the anterior rectal wall; impaired anorectal sphincter relaxation is the cause.
13	A	Juvenile polyps are the commonest hamartomatous polyp; typically in the rectum with bleeding and prolapse.
14	B	Hyperplastic polyps: commonest in the adult colorectum, sawtooth serrations in the upper half of crypts, no conventional dysplasia; malignant risk is questionable.
15	E	Peutz-Jeghers: rare autosomal dominant; multiple GIT hamartomatous polyps (commonest in small intestine); mucocutaneous hyperpigmentation; increased malignancy risk.
16	A	Listed associations: dermatitis herpetiformis, enteropathy-associated T-cell lymphoma, increased small intestinal adenocarcinoma, splenic atrophy.
17	D	On the slide grid Whipple disease is the only disease with a lymphatic defect.
18	B	Abetalipoproteinemia is transepithelial only; celiac and tropical sprue are terminal digestion + transepithelial; CF and chronic pancreatitis are intraluminal; Whipple is lymphatic.
19	E	The cytologic hallmark of adenomatous polyps is epithelial dysplasia. They are benign, give rise to the majority of colorectal adenocarcinomas, but not all progress.
20	D	Villous adenomas are often larger and sessile, covered by slender villi, and more frequently contain foci of invasive malignancy.

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Recalled and source questions are treated as evidence of what gets asked, not as a source of correct answers — verify against your lecture notes.